

## SEPSIS CARE BUNDLE

Does your patient have an EWS more than 3 or you concerned about their condition? Is an infection a possible cause? If so, you need to fill the sepsis Screening Tool which is mentioned below.

| Sl.no. | Resuscitation Bundle                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | YES/NO |
|--------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------|
| 1.     | <b>TO BE COMPLETED WITHIN 1 HR</b><br><br>Measure lactate level<br>Re-measure if initial lactate is $\geq 2$ mmol/L                                                                                                                                                                                                                                                                                                                                                                                                                        |        |
| 2.     | Obtain blood cultures before administering antibiotics<br>Do not delay antibiotics for culture results.<br>Preferably two sets (aerobic and anaerobic) from different sites                                                                                                                                                                                                                                                                                                                                                                |        |
| 3.     | Administer broad-spectrum antibiotics<br>Targeted therapy can be adjusted once pathogen is identified.                                                                                                                                                                                                                                                                                                                                                                                                                                     |        |
|        | <b>FLUID RESUSCITATION</b><br>Begin rapid administration of IV fluids<br>30 mL/kg of crystalloids for hypotension or lactate $\geq 4$ mmol/L.                                                                                                                                                                                                                                                                                                                                                                                              |        |
| 4.     | WITH IN 6 HRS:<br><b>Apply vasopressors if hypotensive during or after fluid resuscitation</b><br>Maintain <b>MAP (mean arterial pressure) <math>\geq 65</math> mmHg</b>                                                                                                                                                                                                                                                                                                                                                                   |        |
| 5.     | <b>RE MEASURE SERUM LACTATE</b> Obtain ABG                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |        |
| 6.     | <b>OXYGEN</b> Hypoxia kills. Gives oxygen via a non-breath the mask at 15L/min                                                                                                                                                                                                                                                                                                                                                                                                                                                             |        |
| 7      | <b>CATHETER</b> <ul style="list-style-type: none"> <li>If BP remains low or if there is evidence of acute kidney injury then consider cauterization.</li> <li>Strict intake/output fluid balance.</li> <li>Aim Urine Output more then 30ml/hr.</li> <li>Acute Kidney injury is a Frequent complication of severe sepsis</li> <li>Continue to monitor EWS at least hrly and still no improvement? Blood pressure failing to improve despite fluid and vasopressor management? Escalated to critical care doctors and consultant.</li> </ul> |        |
|        | <b>Outcome of the sepsis care bundle:</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |        |

Signature of the Senior Doctor:

Signature of the staff:

